

VII.3 Summary

The fixed-dose combinations of **benzoyl peroxide (BPO) plus adapalene** and **BPO plus clindamycin** demonstrate the greatest efficacy against inflammatory acne (II) compared to topical monotherapies.

Monotherapy with **azelaic acid, BPO, or topical retinoids** shows comparable efficacy when directly compared.

Systemic antibiotic monotherapy does not show superiority over topical treatments. Therefore, combining systemic antibiotics with a topical agent is preferred.

For severe acne, **systemic isotretinoin** is recommended due to its well-documented and high efficacy in clinical practice.

Evidence regarding safety and tolerability is very limited and insufficient to guide treatment recommendations.

Due to lack of standardized protocols, clinical experience, and controlled trial data, there is insufficient evidence to support the use of laser or light-based therapies—other than blue light—for papulopustular acne.

VIII Treatment of Nodular/ Conglobate Acne

VIII.1 Recommendations*,†

High strength of recommendation

- **Oral isotretinoin** is strongly recommended as monotherapy for conglobate acne.

Medium strength of recommendation

- **Systemic antibiotics** in combination with **azelaic acid** can be recommended for conglobate acne.

Low strength of recommendation

- **Oral anti-androgens** combined with **oral antibiotics** may be considered for conglobate acne (for female patients only).
- **Systemic antibiotics** combined with **adapalene**, **BPO**, or the **adapalene-BPO fixed-dose combination** may be considered for nodular/conglobate acne.

Negative recommendations

- **Topical monotherapy** is not recommended for conglobate acne.
- **Oral antibiotics** as monotherapy are not recommended.
- **Oral anti-androgens** as monotherapy are not recommended.
- **Artificial UV radiation** is not recommended.
- **Visible light therapy** as monotherapy is not recommended.

Open recommendations

- Due to insufficient evidence, no recommendation can be made for or against the use of **intense pulsed light (IPL)** or **laser therapy** in conglobate acne.
- Although **photodynamic therapy (PDT)** is effective in moderate nodular/conglobate acne, it cannot be routinely recommended due to lack of standardized regimens and concerns about acute adverse reactions.

Limitations such as cost, reimbursement, legal restrictions, drug availability, or licensing may require use of lower-strength treatments as first-line.

†Expert opinion: In the initial phase of isotretinoin treatment, combination with oral corticosteroids may be considered in conglobate acne.*

‡Doxycycline or lymecycline, limited to 3 months.

§Hormonal anti-androgens are indicated only in female patients.

VIII.2 Reasoning

General Comment

Very few clinical trials have specifically evaluated treatments in patients with **nodular or conglobate acne**. As a result, indirect evidence from studies on **severe papulopustular acne** was used, focusing on reductions in **nodule (NO)** and **cyst (CY)** counts. When indirect evidence was used, the strength of recommendation was downgraded accordingly.

VIII.2.1 Efficacy

Superior efficacy was defined as a $\geq 10\%$ difference in outcomes in head-to-head comparisons (see Chapter 3.3.3).

VIII.2.1.1 Systemic Monotherapy vs. Placebo

- **Systemic isotretinoin** demonstrates superior efficacy compared to placebo¹⁹⁰ (Level of Evidence: 4).
- *Note: Only one trial has directly compared isotretinoin with placebo in nodular/conglobate acne, limiting the evidence to Level 4.*